

SILCHAR MEDICAL COLLEGE AND HOSPITAL (SMCH)

Department of Neurology & Neuro-linguistics

Ghungoor, Silchar, Assam 788014

CONFIDENTIAL CLINICAL MEDICAL RECORD - NEUROLOGY CONSULTATION

Patient Name: Mech, Dimple Moni	MRN: SMCH-2026-4029	Sex: Female
DOB / Age: 11/06/1964 (Age: 62)	Date of Exam: 27/08/2026	Primary Caregiver: Bhaben Mech (Husband)
Attending Neurologist: Dr. Anupal Barman, MD, DM (Neurology)	Referring Physician: Dr. S. K. Bhattacharya, MD (General Medicine)	Clinical Status: Outpatient Evaluation

REASON FOR EVALUATION

Assessment for progressive speech non-fluency, severe word-finding pauses, and sentence repetition deficits over 16 months in a relatively young patient.

CLINICAL HISTORY (HISTORY OF PRESENT ILLNESS)

The patient is a 62-year-old right-handed Bodo-Mech female from Silchar, former administrative clerk, presenting with a 16-month history of insidious language deterioration. Her husband describes marked difficulty finding names of common objects, frequent mid-sentence pauses, and phonemic substitution errors. She maintains intact single-word comprehension and object knowledge but cannot repeat multi-clause sentences or multi-digit numbers. Non-verbal memory and everyday visuospatial navigation are preserved. Functional Assessment: Requires assistance for phone calls, banking transactions, complex documentation, and medication schedules. Independent in cooking traditional meals, household cleaning, dressing, and personal hygiene. Avoids driving due to reading street signage difficulties. Behavioral Symptoms: Mild frustration and social withdrawal due to communication barrier; no apathy, disinhibition, or psychotic symptoms.

PAST MEDICAL HISTORY	ACTIVE MEDICATIONS
- Mild Essential Hypertension - No prior stroke, seizures, or developmental language delay	- Amlodipine 5 mg PO daily - Multivitamin daily

PHYSICAL & NEUROLOGICAL EXAMINATION

Alert, cooperative, pleasant. Speech: Fluent conversational rhythm interrupted by prolonged word-finding pauses (anomia) and phonemic paraphasias (e.g., 'plip' for 'clip'). Cranial Nerves: Intact. Motor: Power 5/5 in all limbs, normal tone, no tremors or fasciculations. Reflexes: 2+ symmetric. Gait & Balance: Normal stride, normal arm swing, Romberg negative.

STANDARDIZED COGNITIVE TESTING

Mini-Mental State Examination (MMSE): Score 23/30 | Western Aphasia Battery (WAB) AQ: 72/100

- Orientation: 9/10 (Disoriented only to day of the week)
- Registration: 3/3
- Attention & Calculation: 4/5 (Digit span reverse impaired: 2 digits)
- Recall: 2/3 (Episodic memory preserved)
- Language & Visuospatial: 5/9 (Severely impaired sentence repetition: 0/1; confrontation naming impaired: 3/6; visual construction and Clock Drawing completely intact 10/10)

DIAGNOSTIC WORKUP & NEUROIMAGING BIOMARKERS

- Laboratory: Normal hematologic, metabolic, thyroid, B12, and autoimmune profiles.
- Brain MRI (Without Contrast): Prominent focal asymmetric left posterior perisylvian and left temporoparietal cortical atrophy. Medial temporal structures relatively preserved (MTA Score: Grade 1, Left Parietotemporal Atrophy Grade 3). Minimal ischemic white matter changes (Fazekas Grade 0).
- CSF Biomarkers (where available): Elevated phosphorylated-Tau (p-tau181) and decreased Aβ42/Aβ40 ratio, consistent with underlying Alzheimer's pathophysiology.

DIAGNOSTIC IMPRESSION

- Primary Progressive Aphasia, Logopenic Variant (lvPPA) due to underlying Alzheimer's Disease pathology (ICD-10: G31.01 / F02.80).
- Characterized by prominent word-retrieval anomia, impaired phonological loop/sentence repetition, preserved semantic comprehension, and focal left perisylvian temporoparietal atrophy.

TREATMENT PLAN & CLINICAL RECOMMENDATIONS

- Pharmacotherapy: Initiate Donepezil 5 mg PO daily (escalate to 10 mg at 4 weeks) targeting underlying Alzheimer's tau/amyloid pathology.
- Speech-Language Therapy: Intensive referral for speech rehabilitation focusing on compensatory communication strategies, semantic word-retrieval training, and tablet-based assistive communication apps.
- Family Counseling & Support: Psychoeducation regarding logopenic progressive aphasia; simplify spoken communication at home (short, clear sentences).
- Follow-Up: Multi-disciplinary review in Neuro-linguistic clinic in 3 months.

Electronically Verified & Signed by:

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Report Generated: 27/08/2026 16:30 IST
System ID: OLLAMA-EXTRACT-V2-PERSIST